Healthcare-Acquired Infections – Section Summary

VAP • CRBSI • C. difficile • Influenza • Isolation

Critical Care Board Review • Section 20 • MCQ-Oriented

1. VAP

- Silver-coated ETT ↓ VAP; **subglottic drainage** ↓ VAP
- Late VAP → anti-pseudomonal (**cefepime**); positive culture without signs → continue antibiotics only if indicated

2. Line Infections

- Differential time-to-positivity / quantitative cultures to confirm CRBSI
- VRE → **daptomycin**
- S. aureus CRBSI → **remove catheter**; MSSA → **nafcillin/cefazolin** ≥ 14 d (TEE)
- Gram-negative CRBSI with resistance → carbapenem; replace catheter
- PICC infection → remove + vancomycin + ceftriaxone
- Enterococcus on HD line → treat ± lock, document clearance

3. C. difficile

- Diagnosis → **PCR** for toxin genes
- Fulminant → **oral vancomycin + IV metronidazole**

4. Respiratory & Isolation

- Influenza → **droplet**; end isolation **7 days** after onset
- TB suspected → airborne until diagnosis excluded
- Varicella/zoster → isolate until **all lesions crusted**

5. High-Yield MCQ Pearls

- ☑ S. aureus CRBSI: remove line
- ☑ Zoster: isolate until crusted

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